CARE MAPS

SEE THE CUES. FOLLOW THE LOGIC. PRIORITIZE THE NURSING ACTION.

THE PREPRX CLINICAL FLOW

1. Recognize the cues
2. Identify the immediate problem
3. Assess and prioritize
4. Intervene safely
5. Monitor the response
6. Evaluate: better, worse, or unchanged?

FAST NCLEX RULE

Acute changes, airway/breathing/circulation threats, severe neurologic changes, and unstable vital signs generally outrank chronic or expected findings.

HEART FAILURE / FLUID OVERLOAD

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Decreased cardiac output and excess fluid volume related to impaired ventricular pumping.

1 RECOGNIZE CUES

- Dyspnea/orthopnea
- Crackles; edema; rapid weight gain
- Fatigue or reduced activity tolerance

2 ASSESS / PRIORITIZE

- Airway/breathing first
- Oxygenation, lung sounds, BP/HR
- Compare weight, I&O, perfusion

3 INTERVENE

- Position upright if dyspneic
- Oxygen as ordered/needed
- Give prescribed diuretic; sodium/fluid measures

4 MONITOR

- SpO_2 and respiratory effort
- Urine output, electrolytes, renal function
- Daily weight and edema

5 EVALUATE

- Breathing easier; crackles decrease
- Adequate urine output/perfusion
- Weight/fluid status trends toward goal

NCLEX PRIORITY **New severe dyspnea, hypoxemia, frothy sputum, or acute pulmonary edema requires immediate escalation.**

TRAP Do not focus on ankle edema before a worsening airway/breathing problem.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

ACUTE CORONARY SYNDROME

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Acute pain and decreased cardiac tissue perfusion related to myocardial ischemia.

1 RECOGNIZE CUES

- Pressure/heaviness; may radiate
- Diaphoresis, nausea, dyspnea
- Older adults/women may have atypical symptoms

2 ASSESS / PRIORITIZE

- Rapid ABCs and vital signs
- 12-lead ECG promptly
- Assess pain, perfusion, contraindications

3 INTERVENE

- Activate ACS pathway/rapid evaluation
- Aspirin if ordered and not contraindicated
- Oxygen only when indicated; other meds per orders

4 MONITOR

- Rhythm, BP, SpO_2
- Pain and ECG changes
- Watch for dysrhythmias/shock

5 EVALUATE

- Pain/perfusion improve
- No deterioration or lethal rhythm
- Definitive treatment occurs promptly

NCLEX PRIORITY **Suspected myocardial ischemia is time-sensitive; do not delay ECG/emergency evaluation for a lengthy history.**

TRAP Routine oxygen is not automatically the first intervention when oxygenation is adequate.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

ACUTE STROKE

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Ineffective cerebral tissue perfusion related to interruption of cerebral blood flow.

1 RECOGNIZE CUES

- Sudden facial droop/arm weakness
- Speech, vision, balance changes
- Severe sudden neurologic deficit

2 ASSESS / PRIORITIZE

- Establish last-known-well time
- Rapid neuro assessment and glucose
- Airway/swallow risk; BP and emergency imaging pathway

3 INTERVENE

- Activate stroke response
- Keep NPO until swallow screening
- Prepare for time-sensitive treatment per eligibility

4 MONITOR

- Neuro status and airway
- BP/glucose per protocol
- Bleeding or neurologic worsening

5 EVALUATE

- Neurologic status stabilizes/improves
- Aspiration prevented
- Definitive treatment is not delayed

NCLEX PRIORITY **Time last known well is critical because reperfusion eligibility is time-dependent.**

TRAP Do not give food, fluids, or oral medications before swallowing safety is established.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

COPD EXACERBATION

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Impaired gas exchange and ineffective airway clearance related to airflow limitation.

1 RECOGNIZE CUES

- Increasing dyspnea/wheeze
- Cough/sputum change
- Accessory muscle use or fatigue

2 ASSESS / PRIORITIZE

- Work of breathing and mental status
- SpO_2; lung sounds
- ABG if ordered/severe; infection cues

3 INTERVENE

- Position upright
- Titrate oxygen to ordered target
- Bronchodilators and other prescribed therapy

4 MONITOR

- Respiratory rate/effort
- SpO_2 and mental status
- Response to bronchodilator; CO_2 retention signs

5 EVALUATE

- Less distress/wheeze
- Target oxygenation achieved
- No increasing somnolence/fatigue

NCLEX PRIORITY **Increasing drowsiness, exhaustion, or a 'silent' chest can signal respiratory failure.**

TRAP Do not withhold needed oxygen from a hypoxemic client solely because the client has COPD.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

ACUTE ASTHMA

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Ineffective breathing pattern and impaired gas exchange related to acute bronchoconstriction.

1 RECOGNIZE CUES

- Wheeze, chest tightness, cough
- Tachypnea/accessory muscles
- Unable to speak full sentences

2 ASSESS / PRIORITIZE

- Severity and SpO_2
- Peak flow if feasible
- Watch for diminished breath sounds/silent chest

3 INTERVENE

- Rapid-acting bronchodilator as prescribed
- Oxygen when hypoxemic
- Systemic corticosteroid per treatment plan

4 MONITOR

- Work of breathing
- Breath sounds/SpO_2
- Peak flow and response to treatment

5 EVALUATE

- Air movement improves
- Dyspnea decreases
- No progression toward respiratory failure

NCLEX PRIORITY **A quiet or silent chest in a severely distressed patient is an ominous finding, not improvement.**

TRAP Do not interpret disappearance of wheezing as recovery if air movement is actually worsening.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

PNEUMONIA

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Impaired gas exchange and ineffective airway clearance related to alveolar inflammation/infection.

1 RECOGNIZE CUES

- Fever/cough/sputum
- Crackles, dyspnea, hypoxemia
- Older adults may present with confusion

2 ASSESS / PRIORITIZE

- Oxygenation and respiratory effort
- Lung sounds and hydration
- Cultures/labs as ordered; sepsis screen

3 INTERVENE

- Position to optimize ventilation
- Oxygen as indicated
- Antimicrobials and pulmonary hygiene as ordered

4 MONITOR

- SpO_2, RR, temperature
- Mental status and BP
- Hydration and secretion clearance

5 EVALUATE

- Oxygenation and work of breathing improve
- Fever/infection trend improves
- No sepsis progression

NCLEX PRIORITY **Hypoxemia, hypotension, new confusion, or rising respiratory distress raises priority above routine fever management.**

TRAP Do not prioritize temperature reduction over an unstable airway/breathing/circulation problem.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

SEPSIS / SEPTIC SHOCK

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Ineffective tissue perfusion related to dysregulated systemic response to infection.

1 RECOGNIZE CUES

- Suspected infection plus acute deterioration
- Hypotension/tachycardia/tachypnea
- Altered mentation, low urine output, elevated lactate

2 ASSESS / PRIORITIZE

- Rapid ABCs and perfusion
- Obtain ordered cultures/lactate without delaying therapy
- Identify shock/organ dysfunction

3 INTERVENE

- Activate sepsis pathway
- Prompt antimicrobials and IV fluids as ordered
- Vasopressor support if shock persists per orders

4 MONITOR

- MAP/BP and mental status
- Urine output/lactate trend
- Oxygenation and organ function

5 EVALUATE

- Perfusion improves
- Urine output/mentation improve
- No worsening organ dysfunction

NCLEX PRIORITY **Sepsis with hypotension or organ dysfunction is an emergency; treatment is time-sensitive.**

TRAP Do not wait for culture results before beginning ordered empiric antimicrobial therapy.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

HYPOGLYCEMIA

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Risk for injury and unstable blood glucose related to inadequate circulating glucose.

1 RECOGNIZE CUES

- Sweating, tremor, hunger
- Confusion, irritability, weakness
- Seizure/unconsciousness if severe

2 ASSESS / PRIORITIZE

- Check glucose promptly when possible
- Determine ability to swallow safely
- Assess cause: insulin, missed meal, exertion

3 INTERVENE

- If awake/safe swallow: rapid carbohydrate per protocol
- If unable to swallow: IV dextrose or glucagon per orders/protocol
- Recheck and follow with longer-acting intake when appropriate

4 MONITOR

- Repeat glucose
- Mental status and symptoms
- Watch for recurrence

5 EVALUATE

- Glucose returns to safe range
- Neurologic symptoms resolve
- Cause is addressed

NCLEX PRIORITY **Altered consciousness with suspected hypoglycemia requires rapid treatment; protect the airway and do not give oral intake.**

TRAP Do not give food or drink to an unconscious client.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

DIABETIC KETOACIDOSIS

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Deficient fluid volume and metabolic acidosis related to insulin deficiency and osmotic diuresis.

1 RECOGNIZE CUES

- Hyperglycemia, polyuria, dehydration
- Kussmaul respirations
- Ketones, acidosis; abdominal symptoms

2 ASSESS / PRIORITIZE

- Hemodynamics and hydration
- Glucose, electrolytes, anion gap/pH
- Potassium is crucial before/during insulin therapy

3 INTERVENE

- IV isotonic fluids first per protocol
- IV insulin after potassium assessment per orders
- Replace electrolytes; treat precipitating cause

4 MONITOR

- Glucose hourly per protocol
- Potassium and cardiac rhythm
- Anion gap/pH and neuro status

5 EVALUATE

- Perfusion improves
- Gap closes/acidosis resolves
- Electrolytes remain safe

NCLEX PRIORITY **Potassium may fall rapidly after insulin begins; potassium status can determine whether insulin must be delayed.**

TRAP Do not judge DKA resolution by glucose alone-the ketoacidosis/anion gap must resolve.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

ACUTE KIDNEY INJURY

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Risk for electrolyte imbalance and excess fluid volume related to abrupt decline in renal function.

1 RECOGNIZE CUES

- Rising creatinine; oliguria
- Fluid retention/edema
- Hyperkalemia or uremic symptoms

2 ASSESS / PRIORITIZE

- Trend urine output and creatinine
- Assess volume status and nephrotoxin exposure
- ECG/potassium if hyperkalemia suspected

3 INTERVENE

- Treat cause and optimize perfusion as ordered
- Avoid/adjust nephrotoxins per orders
- Manage fluid/electrolyte complications

4 MONITOR

- I&O and daily weight
- Potassium/acid-base status
- Respiratory status and BP

5 EVALUATE

- Urine output/renal trend stabilizes
- Electrolytes controlled
- No pulmonary edema/dysrhythmia

NCLEX PRIORITY **Hyperkalemia with ECG changes or pulmonary edema can become immediately life-threatening.**

TRAP Do not assume giving more fluid is always appropriate; AKI may coexist with fluid overload.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

HYPERKALEMIA

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Risk for decreased cardiac output related to altered cardiac conduction from elevated potassium.

1 RECOGNIZE CUES

- Weakness/paresthesia may occur
- Elevated potassium
- ECG changes can progress to lethal dysrhythmias

2 ASSESS / PRIORITIZE

- Verify severity and ECG
- Review renal function/medications
- Assess for symptoms and specimen hemolysis if appropriate

3 INTERVENE

- Cardiac monitoring
- Urgent stabilizing/shift/removal therapies as ordered
- Stop potassium sources/causative meds only as directed

4 MONITOR

- Continuous rhythm if severe
- Repeat potassium/glucose as indicated
- Renal status

5 EVALUATE

- ECG stabilizes
- Potassium moves toward target
- No dysrhythmia

NCLEX PRIORITY **ECG changes plus severe hyperkalemia demand immediate action because cardiac arrest risk is high.**

TRAP Do not delay treatment of clinically significant hyperkalemia while focusing only on the lab number.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

HYPONATREMIA

Recognize cues -> prioritize -> intervene -> evaluate

COMMON NURSING PROBLEM

Risk for acute neurologic injury related to low serum sodium and cerebral edema.

1 RECOGNIZE CUES

- Headache, nausea, confusion
- Weakness; severe cases seizures
- Often related to excess water, losses, or SIADH

2 ASSESS / PRIORITIZE

- Neuro status and severity/onset
- Volume status
- Review sodium trend and contributing meds/conditions

3 INTERVENE

- Seizure precautions if severe
- Fluid strategy and sodium replacement per cause/orders
- Correct at a controlled rate

4 MONITOR

- Frequent neuro checks
- Serum sodium trend
- I&O and response to therapy

5 EVALUATE

- Neuro status improves
- Sodium rises safely
- No osmotic demyelination from overcorrection

NCLEX PRIORITY **Seizures or severe neurologic symptoms make this an emergency, but sodium correction must still be controlled.**

TRAP Do not correct chronic hyponatremia too rapidly.

Use clinical judgment and the client's actual condition/orders. These maps are quick-review aids, not substitutes for facility protocols.

NCLEX: What Comes First?

UNSTABLE > STABLE	Acute deterioration, severe abnormal vital signs, new neurologic deficits, or threatened airway outrank routine needs.
AIRWAY / BREATHING / CIRCULATION	Use ABCs when an immediate physiologic threat is present-not as a rigid rule for every question.
ASSESS BEFORE YOU ACT	Assess first when you lack essential data. Act first when the emergency and required response are already clear.
ACTUAL > POTENTIAL	An existing life-threatening problem generally outranks a risk diagnosis.
ACUTE > CHRONIC	A new or unexpected change generally outranks an expected chronic finding.
SAFETY BEFORE ROUTINE	Stop unsafe medication/procedure situations and protect the patient before completing nonurgent tasks.
EVALUATE THE RESPONSE	After an intervention, determine whether the patient improved, worsened, or needs escalation.